

Why Is My Senior Cat Howling at Night?

The CDS, Deafness & Sundown Syndrome Owner's Survival Guide For cats 10 years and older

Introduction: When Nothing You Try Works

Your cat is 12, 14, maybe 16 years old. Somewhere in the past few months, something changed. She started crying at 1 a.m. Then 3 a.m. Then 5. The sound isn't the polite meow she's always had — it's loud, hoarse, almost mournful. She paces. She stares at walls. She can't find her way to her bed even though it's three feet away.

You've tried ignoring it. You've tried the calming plugin. You've tried closing the bedroom door, but then she yowls at the door. You've tried white noise. You've talked to your vet, who ran bloodwork and said everything looks fine.

You're exhausted. You're frustrated. And worst of all — you feel guilty for being frustrated, because she's an old cat and this must be hard for her too.

This guide is for you.

The reason nothing has worked is that you've been given advice meant for young cats with attention-seeking behavior. Your cat isn't trying to manipulate you. She's disoriented. She's confused. She may be in pain. She may be losing her hearing, her sight, her grip on time and space. This is feline cognitive dysfunction — essentially dementia — and it has a specific set of solutions that look nothing like the generic advice you've been given.

This guide covers:

- The medical conditions that cause sudden nighttime yowling in senior cats (and how to catch them)
- How to adapt your home so a confused cat can navigate at night without panicking
- Specific adaptations for deaf cats, cats with vision loss, and cats with arthritis
- How to take care of yourself through this — because caregiver exhaustion is real

What this guide does not do: tell you to "just ignore it." That advice is for a different problem.

Page 2: The Vet Visit That Changes Everything

Before you change anything in your home, before you buy another gadget or plug in another plugin — take your cat back to the vet. Not for another general checkup. For a senior-specific workup with specific tests.

Here's why this comes first: there are medical conditions common in older cats that cause nighttime vocalization. Some are very treatable. If your cat has one of these and you address only the behavior, you're missing the real problem.

What to request at the vet

Call ahead and tell them you're coming specifically to investigate increased nighttime vocalization in a senior cat. Request the following:

Full senior blood panel (chemistry + CBC)

This checks organ function, red and white blood cell counts, and electrolytes. Critically, it includes the T4 thyroid test — more on why that matters below.

Total T4 (thyroid function)

Hyperthyroidism is the most common endocrine disease in cats over 10. The classic signs: weight loss despite a normal or increased appetite, increased activity or restlessness, and yes — increased vocalization, especially at night. Your cat's body is running at metabolic overdrive. It's treatable with medication, radioactive iodine therapy, or a prescription diet. If this is what's driving the yowling, you could see improvement within weeks of starting treatment.

Blood pressure

Hypertension is common in older cats, especially those with kidney disease. High blood pressure causes disorientation, head tilts, circling, and crying out — especially noticeable at night when the house is quiet. It's measured the same way it is in humans — with a cuff on the cat's leg or tail. If blood pressure is high, medication can bring it down and your cat's confusion may clear.

Arthritis screening

Your vet can palpate joints and assess for pain response. Most cats over 12 have arthritis to some degree. Cats are exceptional at hiding pain — they don't limp, they just move less, sleep more, and may cry out when settling into a painful position at night. Pain management (joint supplements, prescription anti-inflammatories, gabapentin) can reduce yowling significantly.

Urinalysis

Rules out urinary tract infections, which cause discomfort and increased vocalization. Especially important in cats who have started urinating outside the litter box.

What "everything looks fine" actually means

If your vet said bloodwork was normal, ask specifically: was the T4 tested? Was blood pressure measured? Was a urinalysis done? Was the cat's joints palpated for arthritis? If those specific tests weren't done, "normal" doesn't rule out the treatable conditions that cause this kind of yowling.

The vet visit checklist

- ☐ Request senior blood panel (chemistry + CBC)
 - ☐ Request Total T4 (thyroid panel)
 - ☐ Request blood pressure measurement
 - ☐ Request joint palpation for arthritis screening
 - ☐ Request urinalysis
 - ☐ Bring a list of all behavior changes you've noticed (not just the yowling — any changes in appetite, litter box use, grooming, where she sleeps, how she navigates)
 - ☐ Note the times of day the yowling is worst
-

Page 3: Medical Deep Dive — What Your Vet Is Looking For

Hyperthyroidism

The thyroid gland sits in your cat's neck. When it becomes overactive, it floods the body with thyroid hormone. This speeds up metabolism, increases heart rate, and makes cats restless and hungry — but it also commonly causes behavioral changes, including loud, persistent vocalization at night.

Affected cats often yowl at 2 or 3 in the morning. They may also eat voraciously while losing weight, drink more water, and groom less. Some become aggressive or anxious. The yowling is not attention-seeking — it's a direct result of the neurological agitation caused by excess thyroid hormone.

Treatment options:

- Methimazole (oral medication): Controls thyroid hormone production. Requires daily dosing and periodic blood monitoring.
- Prescription iodine^{飲食} (Hill's y/d): Iodine-restricted diet that reduces thyroid hormone production. Only works if the cat eats nothing else.
- Radioactive iodine therapy (I-131): A one-time injection that destroys the overactive thyroid tissue. Considered the gold standard cure. Requires 1-2 weeks of hospitalization.
- Surgical thyroidectomy: Removal of the thyroid glands. Less common now due to the risks of anesthesia in older cats.

Cognitive Dysfunction Syndrome (CDS)

CDS is the feline equivalent of Alzheimer's disease. It's caused by the accumulation of beta-amyloid proteins in the brain, leading to neuron death over time. Cornell University researchers estimate that more than 50% of cats 11-15 years old show at least one sign of CDS. By 15, that number increases.

The hallmark signs (DISHA — disorientation, interactions altered, sleep-wake cycle changes, housetraining alterations, anxiety):

- Disorientation: getting lost in familiar rooms, staring at walls or corners, not recognizing family members
- Altered interactions: suddenly aggressive, or suddenly distant, or yowling for no apparent reason
- Sleep-wake cycle changes: sleeping all day, awake and vocalizing all night
- Housetraining lapses: urinating outside the litter box, especially at night
- Anxiety: increased agitation, especially in low-light or unfamiliar situations

The nighttime yowling in CDS is specifically related to the cat's internal clock malfunctioning. The part of the brain that regulates day-night cycles is affected. The cat literally does not know it's nighttime. She may be confused about where she is, what time it is, or why she can't find her family.

CDS is not curable, but it's manageable. The sooner you recognize it, the sooner you can put supports in place.

Hypertension

High blood pressure in cats is usually secondary to kidney disease or hyperthyroidism. The elevated pressure affects the brain, causing disorientation, head tilt, circling, and frequent loud vocalization — particularly noticeable at night.

Diagnosis requires a blood pressure cuff. Treatment is typically a calcium channel blocker or ACE inhibitor, given orally. Once blood pressure is controlled, many cats become noticeably calmer within days to weeks.

Arthritis

X-ray evidence of arthritis is found in the vast majority of cats over age 12, even when they show no obvious limping. Cats with arthritis experience chronic low-grade pain that worsens when they're cold, when they have to jump down from heights, or when they settle into a sleeping position that compresses a painful joint.

Pain can cause vocalization at night because the house is cooler, because the cat has to navigate to her bed (which may require jumping down from furniture), and because the silence amplifies discomfort. Pain also disrupts sleep, making nighttime waking worse.

Management includes: joint supplements (glucosamine, chondroitin, omega-3 fatty acids), prescription anti-inflammatory medications, gabapentin for nerve pain, heated beds, and ensuring all resources are on one floor so the cat doesn't need to climb stairs.

Page 4: Nighttime Environment Makeover

Once you've ruled out or started treating the medical causes, it's time to adapt your home. A cat with CDS or sensory decline needs a nighttime environment designed for her specific impairments. Generic advice like "try a white noise machine" is insufficient. Here's what actually works.

Night lights throughout the home

One of the simplest and highest-impact changes you can make. Cats with CDS or vision loss navigate poorly in the dark. A cat who yowls because she can't find her bed at 3 a.m. may stop yowling once a night light is installed between her favorite spot and her litter box.

Place night lights:

- In hallways your cat uses to get from room to room
- Near the litter box (cats need to find it at night, and many older cats have reduced vision)
- Near food and water stations
- At the top and bottom of stairs
- Near your cat's sleeping area
- If your bedroom door is closed: a light visible from the crack under the door helps a confused cat know the family is still there

You don't need bright lights. Soft plug-in night lights with dusk-to-dawn sensors are ideal. Motion-activated lights in hallways are even better — they provide light when needed without being on all night.

Litter box accessibility audit

A cat with arthritis, CDS, or vision loss may stop using the litter box simply because it's too difficult to reach. An inaccessible litter box is a common trigger for nighttime yowling in senior cats — she's trying to tell you she can't get to it.

Checklist:

- Is there at least one litter box on every floor of your home?
- Is the litter box entry low enough for a cat with stiff joints? (High sides are hard for arthritic cats — consider a box with one low-cut entry side)
- Is the path to the litter box clear of obstacles, other pets, or furniture that requires navigating?
- Is the litter box large enough for a senior cat to turn around comfortably?
- If your cat sleeps upstairs, is there a litter box within one floor of her sleeping area?

Consistent furniture placement

This is critical for CDS cats. If you move furniture — even a chair in the hallway — your cat may lose her spatial map entirely. She'll pace, yowl, and appear lost. Keep everything in the same place. Don't move her bed. Don't put her carrier in a new spot. The environment she knew at 8 years old needs to be the same environment at 16.

Feeding and watering on the right floor

If your cat sleeps upstairs but her food and water are downstairs, she may yowl because she's hungry or thirsty but can't navigate the stairs at night. Move food, water, and at least one litter box to the floor where she spends most of her time.

Pheromone strategy

Feliway Classic: provides general feline calming pheromones. Can reduce anxiety-driven yowling. Plug diffusers in rooms your cat uses most, including near sleeping areas.

Feliway Optimum: newer formulation specifically formulated for CDS-related anxiety and multi-factor calming. If you can only use one, this is the one to try.

Apply pheromones 2-4 weeks before expecting results — they work gradually.

The evening routine that helps

Cats with CDS do better with predictable routines. An evening sequence that ends with settling can reduce nighttime confusion:

- Evening play session (even 5 minutes of gentle wand toy play reduces cortisol and helps with sleep pressure)
- Small late-evening meal or snack (feeding after play simulates the hunt-catch-eat cycle)
- Gentle brushing or petting
- Pheromone diffuser running
- Night lights on throughout the house
- Settle into the same sleeping spot at the same time every night

Page 5: CDS-Specific Comfort Protocol

Understanding the cat's experience

Imagine losing your sense of time so completely that midnight feels like 5 p.m. Imagine not recognizing your living room because the shadows make it look foreign. Imagine being hungry at 3 a.m. and not understanding why the kitchen is dark and no one is there. Imagine being old, stiff, in low-grade pain, and unable to tell anyone.

This is your cat's experience. The yowling is not misbehavior. It's distress.

Understanding this changes how you respond. It doesn't make the sleepless nights easier, but it helps you hold the frustration without guilt.

Environmental anchors

An environmental anchor is a consistent sensory cue that helps a disoriented cat orient herself. These can be incredibly effective for reducing CDS-related yowling.

Scent anchors: Keep a worn t-shirt or sock near your cat's bed. Your familiar scent provides reassurance when she's disoriented. If you sleep separately from your cat, having your scent in her room can reduce nighttime distress yowling.

Audio anchors: Cats find consistent low-level sound soothing. Leave a white noise machine, a radio on low, or a TV playing nature documentaries (with sound) at the same volume every night. The consistency matters — the cat learns to associate that sound with the sleeping hours.

Visual anchors: A night light placed consistently in the same spot becomes a landmark. For severely disoriented cats, a small lamp left on at the same intensity every night can serve as a visual anchor.

What makes it worse

- Moving furniture or changing the environment
- Inconsistent feeding times
- Loud noises (construction, fireworks, thunderstorms) — cats with CDS recover more slowly from sensory disruptions
- Punishment or negative responses — these increase cortisol, worsen anxiety, and teach the cat that nighttime is frightening
- Being left alone for long periods with no environmental stimulation
- Cold rooms — arthritis pain worsens in cold environments; keep your home warm at night

What actually helps

- Consistent, predictable routines
- Environmental enrichment during waking hours (window perches, bird feeders outside windows, puzzle feeders)
- Short, gentle play sessions to build healthy sleep pressure
- Access to all resources (food, water, litter) on the floor where the cat sleeps
- Night lights in all transition spaces
- Pheromone diffusers

- Heated beds (for arthritic comfort)
-

Page 6: Adaptations for Deaf Cats

Deaf cats present a unique challenge because their yowling is different from CDS-driven yowling. A deaf cat yowls louder partly because she can't hear herself. The yowling may also be a social call — she's trying to communicate and can't hear a response.

Deaf cats rely entirely on visual and vibrational cues. Here are the adaptations that help:

Visual cue training

Teach your deaf cat basic visual signals. Use a flashlight with a soft glow (not a laser) as a recall cue — flash it two or three times to get her attention, then use a hand signal. You can also use a vibrating collar (not an e-collar, just a collar with a small vibrating motor) as a general "look at me" cue.

The flashlight signal is particularly useful at night. A brief flash directed toward your cat, followed by a treat, builds the association. At night, this can replace verbal calling.

Vibration-based routines

Deaf cats can't hear the sound of an automated feeder, but they can feel the vibration of it dispensing food. Place the feeder on a hard surface near your cat's sleeping area so she can feel the vibration when food is dispensed. This teaches her to associate the feeder with mealtime.

Night safety

A deaf cat who slips outside at night can't hear cars, dogs, or people approaching. If you let a deaf cat outside, consider converting to indoor-only with a secure catio or supervised outdoor time. If you must let her out, consider a collar with a GPS tracker and your contact information.

Touch as communication

Deaf cats rely heavily on touch. A gentle hand on the shoulder or side is a "look at me" signal. Petting in the same direction every time is calming. A gentle pat on the head followed by a treat is positive association.

Why the yowling is louder

A deaf cat's internal volume control is partly auditory — when you can't hear yourself, you don't know you're being loud. Combine this with CDS (common in elderly deaf cats) and you have a cat who is genuinely unaware of how loud she's being.

Solutions:

- Night lights help reduce the disorientation that drives the yowling in the first place
 - A vibrating cat bed or mat may provide enough sensory input to settle her
 - In severe cases, gabapentin prescribed by your vet can reduce the anxiety driving the yowling
 - Thundershirt or anxiety wrap: the gentle pressure is calming for many cats
-

Page 7: The Owner Survival Guide

You're not a bad owner

This needs to be said plainly. You are not failing your cat because you're exhausted. You are not failing her because some nights you put in earplugs just to function the next day. You are caring for a sick, elderly animal, and it is hard.

Feline cognitive dysfunction is a disease. The yowling is a symptom of that disease. Your frustration is a normal human response to chronic sleep deprivation and helplessness. It doesn't mean you love your cat less. It means you need support.

Sleep deprivation survival

The average caregiver for a cat with CDS loses 2-4 hours of sleep per night. This compounds quickly. After two weeks, your reaction time is equivalent to someone over the legal alcohol limit. After a month, cognitive function is measurably impaired.

You cannot care for your cat effectively if you're destroyed. Here are the non-negotiable steps:

Earplugs are not failure. They are a medical accommodation for you. Use them.

Rotate sleeping arrangements. If your partner is available, take turns being on "cat duty" overnight. One night you, one night them.

White noise for you, not just the cat. A white noise machine near your bed masks the yowling sound enough to help you stay asleep.

Accept help when it's offered. If a friend or family member offers to sit with your cat for an evening so you can sleep, say yes.

Talk to your vet about medication for the cat. Gabapentin, amitriptyline, or other prescriptions can reduce nighttime anxiety enough to give everyone better sleep. This is not a last resort — for many cats with CDS, it's the intervention that restores household sanity.

Recognizing decline milestones

CDS progresses. Here's how to track it:

Early stage: Occasional nighttime yowling, mild disorientation, occasional litter box misses. Cats usually still interactive, still eating well.

Middle stage: Frequent nighttime yowling, more pronounced disorientation, getting lost in familiar rooms, reduced grooming, altered appetite. More frequent vet visits often needed.

Late stage: Constant disorientation, no longer recognizes family members, incontinence, not eating without assistance, spends most of the day hiding. Quality of life becomes the primary concern.

Track these changes with a simple journal. Record: appetite, litter box use, yowling frequency (scale of 1-10), grooming, interactions with family, and good hours vs. difficult hours each day. This data helps your vet adjust treatment and helps you recognize when you've entered a new phase.

Euthanasia timing

This is the hardest part. No one wants to think about it, but for cats with advanced CDS, it's a real and often necessary consideration.

The question to ask yourself: Is she having more bad hours than good? Is she still experiencing joy, or is she primarily in distress? Can she eat, drink, and eliminate with reasonable comfort?

CDS alone is not a reason to euthanize. Many cats with CDS live comfortably for months or years with the right support. But when a cat is no longer able to rest without constant distress, when she's falling frequently, when she's hiding and crying constantly, when she no longer recognizes her family — those are the conversations to have with your vet.

Quality-of-life scales (like the HHHHHIMMS scale) provide a structured way to evaluate this. Your vet can walk you through it.

Page 8: Quick-Reference Checklist

Vet Visit Checklist

- ☐ Schedule senior blood panel (chemistry + CBC)
- ☐ Request Total T4 thyroid test specifically
- ☐ Request blood pressure measurement
- ☐ Request arthritis palpation exam
- ☐ Request urinalysis
- ☐ List all behavior changes noticed in the past 6 months
- ☐ Note worst yowling times for the vet record

Night Environment Setup Checklist

- ☐ Install night lights in all hallways and transition spaces
- ☐ Add night light near litter box
- ☐ Add night light near food and water stations
- ☐ Add night light near cat's sleeping area
- ☐ Move food/water to the floor where cat sleeps
- ☐ Add at least one litter box to the floor where cat sleeps
- ☐ Audit litter box entry height — switch to low-entry box if needed
- ☐ Check furniture hasn't been moved in the past month
- ☐ Plug in Feliway Optimum diffuser in main living area
- ☐ Establish consistent evening feeding time
- ☐ Set up white noise machine near cat's sleeping area

Quality-of-Life Tracking Checklist

- ☐ Start daily journal: appetite, litter box, yowling score, good hours vs. difficult hours
 - ☐ Review journal weekly for patterns
 - ☐ Note any sudden decline — schedule vet visit
 - ☐ Keep HHHHHIMMS quality-of-life scale accessible for reference
 - ☐ Discuss late-stage care plan with vet before you need it
-

Page 9: Next Steps

You now have a plan. Here's how to move forward.

This week: Book the vet appointment and request the senior-specific tests listed on page 2. Start your nighttime light audit today — pick up a \$6 plug-in night light and install it in one high-traffic feline pathway.

This month: Work through the environment checklist. Install all night lights. Move food and litter boxes to the right floors. Set up the pheromone diffuser and give it two weeks to build effect.

Ongoing: Track your cat's quality of life daily. Adjust interventions as needed. Get sleep yourself — you can't help her if you're destroyed.

Your Bonus: Included

This guide's companion checklist — the Quick-Reference Checklist you see on page 8 — is available as a standalone printable document in your Lemon Squeezy library.

[Lemon Squeezy Library Link] — Your purchased products, organized and ready to download.

If you haven't purchased anything yet and want the printable checklist:

<https://mrohitth.lemonsqueezy.com/checkout/buy/cat-sundown-syndrome-guide>
— Access the full guide and companion checklist.

This guide is for informational purposes only and does not replace veterinary advice. If your cat is yowling, vocalizing in distress, or showing any sudden behavioral changes, consult a veterinarian.